

OMM Lab: Still Technique - Cervical, Lumbar, Thoracic, Innominate & Ribs

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Please be sure to review the material in the “Introduction to Still Technique” lecture before coming to the lab. In that lecture you will learn a brief history of the technique and its basic principles, as well as the indications and contraindications for the Still Tech. There is also a sample case.

Session Objectives

- Apply the principles of the Still Technique to perform the technique on somatic dysfunctions in the regions listed on the next slide.
- Perform the techniques of the review material from FOM Lab #7 Rib CS and HLVA
- Perform the review techniques of the review material from FOM Lab #12 Anterior lumbar CS and lumbar MET
- Understand how to integrate these techniques into a clinical scenario

New Techniques in this lab:

Still technique to the following regions/dysfunctions:

- OA
- AA
- C2-7
- Rib 1 inhalation and exhalation (seated)
- T1-T12
- L1-L5
- anteriorly and posteriorly rotated innominates.

- The techniques are all described in the handout and the posted videos.
- In this brief lecture, we will be discussing a clinical scenario in which you may apply many of the techniques you will learn.

Case

A 40-year-old man presents to your office complaining of neck and back pain after being involved in motor vehicle crash two weeks ago. He initially felt fine after the accident but was taken to the ED where they did cervical spine and lumbosacral spine x-rays which were negative.

Case (continued)

A few days later he noticed pain in his neck, upper back and lower back. The neck pain and upper back pain are sharp, bilateral, and worse when he turns his head to the left. The pain is non-radiating. He has no associated numbness, tingling, or weakness in his arms or hands. Neck pain is 7/10 in severity. Upper back pain is 5/10 in severity.

Case (continued)

Lower back pain is bilateral, constant, worse on the right, and better with flexion, worse with extension. He has no radiation of the pain and no associated numbness, tingling, weakness, or bowel or bladder function changes.

Case (continued)

PMHx: seasonal allergies

PSHx: none

Medications: Loratadine 10 mg daily

Soc Hx: denies drug, alcohol, tobacco, caffeine use.
Used to run 3 miles daily, but has been unable to
since the accident.

Fam Hx: Mother had an MI (heart attack) at age 67

Case (continued)

Physical Exam:

HR:58, Resp:12, Temp: 97.6

Upper and Lower Extremity Motor 5/5 B/L

C5, C6, C7, L4 and S1 Deep tendon reflexes 2/4 B/L

No gross sensory deficit in upper/lower extremities

Negative Spurling's Maneuver

Negative straight leg raise B/L

Negative costovertebral angle tenderness B/L

Case (continued)

Physical Exam (continued):

2+ radial and pedal pulses B/L

Decreased gross cervical ROM to extension,
rotation left and sidebending left

B/L hypertonic SCM mm.

B/L cervical paravertebral muscle spasm (PVMS)

Case (continued)

Physical Exam (continued):

Increased skin drag and positive red reflex test over T1 and T5 on the right

PVMS T1-T5 on right

Decrease translation of T12 and L1 region from right to left

B/L Lumbar PVMS L1-L5

B/L Psoas hypertonicity

Case (continued)

Physical Exam (continued):

OA E SI Rr

AA Rr

C2 ERS r, C7 ERS r

B/L Rib 1 exhaled

T1 ERS r, T5 ERS r,

B/L Respiratory diaphragm hypertonicity

Case (continued)

Physical Exam (continued):

L1 ERS I

RoR FST

Right posteriorly rotated innominate

Case (continued)

Assesment:

1. Whiplash injury
2. Cervicalgia
3. Low Back pain
4. Muscle spasm
5. Somatic Dysfunction – Head
6. Somatic Dysfunction –Cervical
7. Somatic Dysfunction – Thoracic
8. Somatic Dysfunction – Ribs
9. Somatic Dysfunction – Lumbar
10. Somatic Dysfunction – Sacrum
11. Somatic Dysfunction – Pelvis
12. Somatic Dysfunction – Other
(Respiratory Diaphragm)

Case (continued)

Plan:

Whiplash/Cervicalgia – ice/heat as needed, cervical ROM stretches, proper ergonomics (sitting, computer work), consider Physical therapy in no improvement, modify sleeping position to keep neck relaxed; ibuprofen or acetaminophen as directed and as needed

Low back pain – psoas stretch, do not sit more than 45 minutes without taking a break; exercise as tolerated but start slowly and build back up.

Pillow between knees if sleeping on side

Somatic Dysfunctions – OMT as described in the following slides. Drink plenty of water, may feel sore in the next day or two.

Other – Proper sleep hygiene

Case (continued)

Discussion of OMT approach:

Treat legs (probably other dysfunctions)

Treat Psoas' - MET or BLT

Treat L1 ERS I – Still Tech, MET or BLT

Treat Posterior innominate – Still tech

Treat Sacrum (If still there) – MET or BLT

Treat Resp. Diaphragm – (treating psoas might have helped), Doming of diaphragm, treat rib 12 B/L, BLT

Case (continued)

Discussion of OMT approach:

Treat Rib 1 B/L – Still tech, BLT

Treat SCM – look for CS points and treat with CS, could try MET, BLT

Treat T1 – BLT, MET, FPR, Still Tech (sometime the rib 1 holds it up, sometimes you need to treat T1 to get ribs to move)

Treat T5 – Still Tech, MET, HVLA

Treat OA E SI Rr, AA RI, C2 ERS r, C7 ERS r – Still tech, MET using oculocervical reflex

* Sometimes you might need to treat in different order depending on if a dysfunction in one area is creating or holding up another in a different area

Summary

Still Technique:

- You may use the Still technique, if indicated, along with any other indicated technique you know to treat most patients.

Core References

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College of Osteopathic
Medicine

DiGiovanna, Amen and Burns. An Osteopathic Approach to Diagnosis and Treatment, 4th Ed

[Chapter 16: Still Technique](#)

Foundations of Osteopathic Medicine 5th ed.

[Ch 56 Still Technique: A Facilitated Indirect Then Direct Method](#)

Lecture Feedback Form:

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<https://comresearchdata.nyit.edu/redcap/surveys/?s=HRCY448FWYXREL4R>